Twelve Everyday Things to Notice About How Your Mother's Week Goes

What to notice on an ordinary visit and how to write it down, without deciding what any of it means.

You visit your mother most weeks and you notice small things. You're never sure which ones are worth writing down. This guide names twelve everyday things to notice on an ordinary visit and, for each, what to write.

Notice and record. Don't interpret. Nothing here is a test of her or of her home, and a note isn't a finding. What a note means is a question for her doctor, her pharmacist or another professional who knows her.

It's her home, so notice with her and not around her. Tell her what you're writing down, and ask her about each of these. She knows more than one visit can show. The places to look around the house follow the CDC's home checklist for older adults and its brochure for family caregivers. Keeping a medicine list and taking questions to her pharmacist or prescriber follow MedlinePlus, and asking the helpers follows AARP's planning guide. The rest, including what to notice about the pill box and the wording about what to write down, is the Family Care Roadmap's own.

Getting around

1 Walking through the house [1] [2]

NOTICE Whether she seems steady on her feet, and whether she has to walk around furniture, throw rugs or cords to cross a room.

WRITE DOWN What you saw, in which room, and the day: “Went around the coffee table to reach her chair, Sunday.”

2 Stairs and steps [2]

NOTICE The stairs she uses, indoors and out: anything left on them, a broken or uneven step, a loose handrail or one on only one side, a light and a switch at the top and the bottom.

WRITE DOWN Which stairs, and what you saw there. Not whether they're safe.

3 A fall, or a worry about falling [1]

NOTICE Anything she tells you about a fall, and anything she says about being worried about falling.

WRITE DOWN Her words, the date and where it happened. This one also goes to her healthcare provider; see the box below.

Around the house

4 Light after dark [2]

NOTICE Whether the light near the bed is easy to reach, and whether the way from the bed to the bathroom is lit.

WRITE DOWN What's there now: the lamp, the switch, a night light or none.

5 The kitchen [2]

NOTICE Whether the things she uses often are kept on high shelves, and whether she uses a step stool to reach them.

WRITE DOWN What she reaches or climbs for, and what the step stool is like.

6 The bathroom [2]

NOTICE Whether the tub or shower floor is slippery, and whether she uses some support getting in and out of the tub or up from the toilet.

WRITE DOWN What she holds on to, if anything. Ask her; she knows better than you can see.

Medicines

7 The medicine list [3] [1]

NOTICE Whether there's an up-to-date written list of all her medicines and supplements and any allergies.

WRITE DOWN Where the list is kept, its date, and who keeps it current. If there isn't one, write that.

8 How the pills get handled [3]

NOTICE Who fills the pill box, who picks up the prescriptions, and anything that looks different from her usual routine, such as pills still in the slot for a day that's passed.

WRITE DOWN What you saw and the date. Change nothing. The question goes to her pharmacist or her prescriber.

The help around her

9 Who comes, and when

NOTICE The help that arrives in an ordinary week: your sister's Tuesday drive, the neighbor's look-in most mornings, your Sunday.

WRITE DOWN One week of who came, on which day, and what they did.

10 The hours she's on her own

NOTICE The parts of the day and the night when nobody is with her or looking in.

WRITE DOWN The days and the times, as plain facts. How much time alone suits her isn't something to judge from a page.

11 How the helpers are doing [4]

NOTICE What each helper says about their own end of it: what's easy, what they'd like to keep doing, what they'd rather not take on.

WRITE DOWN Their words, and whether anyone has said they could cover for them for a week.

12 What a professional has already said

NOTICE Any instructions her doctor, pharmacist or therapist has already given: a printed sheet, a note on the fridge, something she was told at an appointment.

WRITE DOWN What it says, who said it and when, and where the paper is. Don't add to it or explain it.

When a note isn't enough

Anything on this list that worries you on the day you see it is a call to her doctor's office, not a note for later. That includes a fall, her telling you she's worried about falling, or her seeming unsteady: the CDC's brochure for family caregivers says to tell a healthcare provider right away. Describe what you saw and leave the rest to them. An emergency is a call to emergency services.

What this guide doesn't do

It doesn't say what any of these mean, whether your mother's home is safe, or whether the help around her is enough. Those belong to the professionals who assess her, and your notes are something they can start from.

Ready for the next step?

You've noticed, and you've written it down. The care interview asks about supervision, falls, medication handling, the home and each helper's capacity in a set order, so the picture doesn't depend on what happened to catch your eye on a Sunday, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Centers for Disease Control and Prevention, Family Caregivers: Protect Your Loved Ones from Falling (STEADI brochure, 2018)
2. Centers for Disease Control and Prevention, Check for Safety: A Home Fall Prevention Checklist for Older Adults (STEADI brochure, 2018)
3. MedlinePlus (National Library of Medicine), Caregiving - medication management: MedlinePlus Medical Encyclopedia
4. AARP, Prepare to Care: A Planning Guide for Families